INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Nivolumab and ipilimumab for advanced melanoma

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
21 days
See the active treatment phase

Treatment stages
2
Explained in the treatment reference

Regimen source
NCCP 00431
Version 11

Reference treatment cycle

Treatment stages

01 **Combination induction**
Up to four visits

02 **Nivolumab alone**
Maintenance

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name

Hospital / oncology team

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

SACTCHECK · CHAPTER 01

Understanding your treatment

Nivolumab and ipilimumab for advanced melanoma · National Cancer Control Programme (NCCP) 00431

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Immunotherapy can cause inflammation in healthy organs during treatment or after it stops. Contact the oncology team promptly for new diarrhoea, blood in stools, cough or breathlessness, yellow skin or eyes, severe headache, vision changes, marked thirst, unusual weakness or a spreading rash. Tell any clinician that you are receiving or have received immunotherapy.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

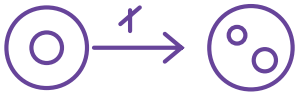
Medicines in this regimen

Nivolumab and ipilimumab help immune cells recognise and attack melanoma. The combination phase is followed by nivolumab alone when appropriate. The aim in this setting is to control advanced disease; benefit and toxicity are reviewed for the individual.

PD-1 CHECKPOINT IMMUNOTHERAPY

Nivolumab

Opdivo



Releases a PD-1 immune brake

Nivolumab prevents the PD-1 checkpoint from dampening T-cell activity. This can strengthen an immune attack on melanoma. It can also trigger inflammation in healthy organs, including after treatment has ended.

PD-1 blocked → Immune activity increases → Healthy organs can inflame

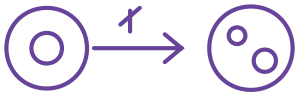
What to watch for

Carry your immunotherapy alert card and report new symptoms early, including after treatment ends.

CTLA-4 CHECKPOINT IMMUNOTHERAPY

Ipilimumab

Yervoy



Releases a second immune brake

Ipilimumab blocks the CTLA-4 checkpoint that limits T-cell activity. In this plan it is combined with nivolumab during induction. The stronger immune effect can also increase inflammation in healthy organs.

CTLA-4 blocked → Combined immune activation → Four-dose induction phase

What to watch for

New diarrhoea, abdominal pain, severe headache or unusual weakness needs early contact. Do not wait for the next infusion.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Understanding your diagnosis and treatment goal

Use these questions with your own pathology report and agreed treatment plan.

What the diagnosis says

Record the original melanoma site and pathology findings. Breslow thickness and ulceration describe features of the skin tumour; stage describes its extent. Melanoma arising in the eye or a mucosal surface may need a different pathway.

BRAF and other tumour tests

BRAF V600 testing can identify an alternative targeted-treatment option in some melanomas. It is separate from the immunotherapy described here. Ask what your tumour result means for the choices available to you.

Surgery, radiotherapy and medicine have different roles

Ask whether medicine is intended to reduce recurrence after complete surgery or control cancer that remains. Add any operation or radiotherapy to the plan with its own purpose, timing and risks.

Stage and response are different

Stage describes the extent of disease. Response describes a change in measurable cancer during treatment. After complete removal there may be no visible tumour to measure; follow-up then looks for recurrence and treatment effects.

A new skin change still needs review

Report a new or changing mole, a sore that persists, a new lump or an unexplained symptom. A photo or a good response in one area cannot rule out a separate problem.

Your medicines and the overall plan

The schedule in this setting

The combination phase gives nivolumab followed by ipilimumab by intravenous (IV) infusion every three weeks for up to four doses. If appropriate, nivolumab alone then continues on the team-selected two-weekly or four-weekly schedule, by the prescribed IV or subcutaneous route. After the last combination dose, the first two-weekly maintenance dose starts three weeks later; the first four-weekly maintenance dose starts six weeks later in this melanoma schedule. Treatment continues until progression or unacceptable toxicity, subject to review.

How the treatment works

Nivolumab prevents the PD-1 checkpoint from dampening T-cell activity. This can strengthen an immune attack on melanoma. It can also trigger inflammation in healthy organs, including after treatment has ended. Ipilimumab blocks the CTLA-4 checkpoint that limits T-cell activity. In this plan it is combined with nivolumab during induction. The stronger immune effect can also increase inflammation in healthy organs.

The role of earlier or later treatment

Ask how the medicine fits with previous or planned local treatment. The timing of an operation, radiotherapy and a tablet course should be written in one agreed plan.

Pauses and stopping

A safety pause is an active treatment decision. Record which medicine is paused, the reason, the tests or recovery needed and who may authorise restarting. Do not restart from an old prescription or reset the history when a new phase begins.

Questions, decisions and practical support

Your priorities and the burden of treatment belong in the discussion.

Benefits and uncertainties

Ask what outcome this treatment aims to improve for your diagnosis and what alternatives exist. Results from a study group help a discussion but cannot promise your own result.

Alternatives and review points

Agree the next decision point and what would make the plan change. Ask about other suitable treatments, trials and symptom care. Support for comfort and function can continue alongside cancer treatment.

Help with daily life

Tell the team what is hardest in daily life: travel, work, caring duties, costs, sleep or confidence. Ask who can help and how to contact them.

Shared understanding

Before going home, check that you can describe your medicine schedule, next tests and urgent-contact route. Ask for an interpreter, larger print or a supporter if that would help.

What the evidence means for your treatment

Discuss the evidence for your diagnosis, tumour test results and treatment setting.

Ask which outcomes matter in your setting and what benefit is realistic for you. No survival percentage from another regimen, dose combination or disease setting is assigned here.

SACTCHECK · CHAPTER 02

Your schedule, tests and appointments

Nivolumab and ipilimumab for advanced melanoma · NCCP 00431

The overall treatment plan

Treatment stages

01 **Combination induction**
Up to four visits

02 **Nivolumab alone**
Maintenance

Your team confirms the active stage and dates. The full treatment guide explains each step.

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

UP TO FOUR VISITS

Combination induction

Nivolumab then ipilimumab | **Every 21 days, up to four doses**

Nivolumab 1 mg/kg and ipilimumab 3 mg/kg are given by intravenous (IV) infusion, subject to review before every dose. Four is the planned combination limit, not the total number of all treatment visits.

MAINTENANCE

Nivolumab alone

Every 2 or 4 weeks as prescribed | **IV or subcutaneous route as selected**

After the last melanoma combination dose, the first two-weekly nivolumab dose starts three weeks later or the first four-weekly dose six weeks later. The team confirms dose, route, interval and start date. Treatment continues while beneficial and tolerable; previous immune-toxicity decisions still apply.

The treatment setting determines duration; a reference calendar is not a prescription or a reason to restart a held medicine.

BASELINE → TREATMENT → FOLLOW-UP

Required safety checks and your individual disease-assessment plan

The team supplies actual dates and resolves any source ambiguity. A reference period does not replace a prescription, a local monitoring plan or an individual treatment decision.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00431 v11: current regimen source

THE WHOLE TREATMENT JOURNEY

Nivolumab and ipilimumab for advanced melanoma: treatment and checks

NCCP 00431a: advanced unresectable or metastatic melanoma in adults. This patient guide is the melanoma context, not the separate liver-cancer indication.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	During treatment	Between visits	Response / follow-up	After a hold or stop
Bloods and organ safety NCCP 00431	Full blood count, kidney and liver tests, glucose and thyroid tests; BRAF tumour testing and review of autoimmune conditions, transplantation, prior immune reactions and current medicines.	Full blood count, kidney/ liver tests, glucose and thyroid tests each treatment cycle. Cortisol and other tests are added when clinically indicated. Every visit also reviews bowel, breathing, skin, endocrine, neurological and other symptoms; a normal blood test does not exclude immune toxicity.	Contact the team for new symptoms; extra tests may be needed between scheduled checks.	Safety bloods do not by themselves show whether cancer is responding.	Continue to report possible immune effects for at least five months after the last dose, as specified by NCCP, and for longer whenever your team advises. Keep any steroid taper, hormone-replacement plan and relevant follow-up tests. Response imaging follows your individual plan.
Cancer, skin and daily function NCCP 00431	Record diagnosis, stage, current disease sites and the baseline assessment.	Keep the source-specific skin and clinical reviews and the individually agreed response assessments.	Record new lesions, lumps, wound changes, pain and effects on usual activities.	Write the next examination or scan date and the responsible team; no universal scan or tumour-marker interval is assumed.	Your stage, symptoms and treatment determine follow-up. Write down the skin and lymph-node checks, any body or brain scans and the service arranging them. Confirm your own dates with the team.

Care / check	Before starting	During treatment	Between visits	Response / follow-up	After a hold or stop
Medicines and continuing precautions NCCP 00431	Review all medicines and the pregnancy/ fertility plan.	Review treatment taken or received, holds, adverse effects and supportive medicines.	Keep the actual urgent contacts and do not restart a held medicine yourself.	Bring the medicine list and treatment summary to other services.	Discuss fertility and pregnancy precautions before starting. Immunotherapy may harm a pregnancy. Use the contraception plan and after-treatment duration specified for the medicines you receive; ask about breastfeeding restrictions. Tell the team promptly about possible pregnancy. Do not stop necessary hormone replacement or prescribed steroids without advice.

What each check is for

1 Combination induction

Up to four visits

Nivolumab and ipilimumab help immune cells recognise and attack melanoma. The combination phase is followed by nivolumab alone when appropriate. The aim in this setting is to control advanced disease; benefit and toxicity are reviewed for the individual.

Ask / **act:** Confirm the current treatment phase, prescription, tests and support medicines.

2 Nivolumab alone

Maintenance

Nivolumab and ipilimumab help immune cells recognise and attack melanoma. The combination phase is followed by nivolumab alone when appropriate. The aim in this setting is to control advanced disease; benefit and toxicity are reviewed for the individual.

Ask / **act:** Confirm the current treatment phase, prescription, tests and support medicines.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Taking or receiving treatment

At each combination visit, nivolumab is given before ipilimumab, as separate IV infusions. The source schedule gives each over 30 minutes; allow extra time for assessment, preparation and observation. Maintenance may be IV or subcutaneous nivolumab with its own dose and appointment interval. The route and interval are recorded in your plan; they are not interchangeable on your own.

STEP 2

Missed, vomited or held treatment

If an appointment is missed or treatment is held, contact the unit for a new plan. Do not count a missed visit as a dose received. A move to maintenance does not erase previous immune toxicity or a permanent-stop decision.

STEP 3

Appointments and supply

Plan blood tests, travel and medicine collection in advance. If you need an interpreter or someone to help at home, tell the team before the appointment.

STEP 4

Paper copies and contact information

Ask for your latest schedule and contact details on paper. Replace old versions when treatment changes so everyone helping you uses the same plan.

Taking treatment and preparing for visits

Keep the written medicine plan beside this guide; it records your own dose and dates.

Taking or receiving the medicine

Other medicines and supplements

Show the pharmacist a current list of prescriptions, over-the-counter medicines, vitamins, herbal products and supplements. Include anticoagulants, heart medicines, steroids and recent antibiotics. Check before adding, stopping or changing anything; an apparently harmless product can alter treatment or confuse a symptom assessment.

Practical preparation

Bring your medicine list, allergy information and questions. Tell the team about practical problems with travel, swallowing, wound care or remembering doses before treatment starts.

Follow-up and your continuing record

Keep this record when treatment is paused, changed or completed.

Your follow-up plan

Your stage, symptoms and treatment determine follow-up. Write down the skin and lymph-node checks, any body or brain scans and the service arranging them. Confirm your own dates with the team.



Continuing medicine precautions

Continue to report possible immune effects for at least five months after the last dose, as specified by NCCP, and for longer whenever your team advises. Keep any steroid taper, hormone-replacement plan and relevant follow-up tests. Response imaging follows your individual plan.

Treatment summary for other clinicians

Record the medicines and last dates, important reactions, any long-term hormone replacement or steroid taper, and the responsible oncology contact. Show this information in urgent care and to other specialists. A different treatment phase does not erase earlier toxicity.

The complete paper version

Use this handbook as your paper record. Keep the current schedule, medicines, tests and contacts together, and replace outdated pages after a change. The matching QR link is optional.

Symptoms and supportive care

Nivolumab and ipilimumab for advanced melanoma · NCCP 00431

Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Combination infusion	 **Day 1** Treatment after clinical review and the required tests.
Symptoms between visits	 **Days 1–21** Immune effects can begin at any time, including after treatment stops.
Next-cycle assessment	 **Day 21** The team confirms the next dose; maintenance has its own schedule.

Timing note 1: The combination phase gives nivolumab followed by ipilimumab by IV infusion every three weeks for up to four doses. If appropriate, nivolumab alone then continues on the team-selected two-weekly or four-weekly schedule, by the prescribed IV or subcutaneous route. After the last combination dose, the first two-weekly maintenance dose starts three weeks later; the first four-weekly maintenance dose starts six weeks later in this melanoma schedule. Treatment continues until progression or unacceptable toxicity, subject to review.

Timing note 2: If an appointment is missed or treatment is held, contact the unit for a new plan. Do not count a missed visit as a dose received. A move to maintenance does not erase previous immune toxicity or a permanent-stop decision.

Timing note 3: This is nivolumab1/ipilimumab3, not the lower-ipilimumab combination.

Timing note 4: Four induction doses do not mean the whole treatment course is four doses.

Timing note 5: Prior immune toxicity and permanent-stop decisions remain relevant when maintenance starts.

Timing note 6: No routine percentage dose reductions: the team may hold or stop immunotherapy and treat the adverse effect.

Why these bands stay broad: This calendar shows the pattern. Your team supplies actual dates, prescribed doses and any hold or restart instructions. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00431 v11: current regimen source ↗

There are no guaranteed "safe days". The prescribed dose, treatment decision and local urgent-contact plan take priority. Call 112 or 999 for severe breathing difficulty, severe chest pain, collapse, a new seizure, sudden weakness or speech difficulty, or uncontrolled bleeding. Follow your oncology unit's written urgent-contact plan for other new or worsening symptoms; do not wait for the next appointment.

Between-treatment notes

Day 1: Treatment visit

At each combination visit, nivolumab is given before ipilimumab, as separate IV infusions. The source schedule gives each over 30 minutes; allow extra time for assessment, preparation and observation. Maintenance may be IV or subcutaneous nivolumab with its own dose and appointment interval. The route and interval are recorded in your plan; they are not interchangeable on your own. Report symptoms and previous reactions before treatment.

Day 2–21: Between visits

Immunotherapy can cause inflammation in healthy organs during treatment or after it stops. Contact the oncology team promptly for new diarrhoea, blood in stools, cough or breathlessness, yellow skin or eyes, severe headache, vision changes, marked thirst, unusual weakness or a spreading rash. Tell any clinician that you are receiving or have received immunotherapy. Do not wait for the next infusion to report a possible immune effect.

Support around the medicines

1 An individual symptom plan

Ask which anti-sickness or other symptom medicines are prescribed, when to take them and when to call if they do not help. Do not start a borrowed medicine or alter treatment to manage a side effect yourself.

2 Your urgent contacts

Keep your actual oncology, out-of-hours and pharmacy contacts on the wallet card. Example numbers on a template must be replaced before you use it.

3 Pregnancy and fertility precautions

Discuss fertility and pregnancy precautions before starting. Immunotherapy may harm a pregnancy. Use the contraception plan and after-treatment duration specified for the medicines you receive; ask about breastfeeding restrictions. Tell the team promptly about possible pregnancy. Do not stop necessary hormone replacement or prescribed steroids without advice.

4 Eating, activity and practical help

Let the team know if cramps, altered taste or poor appetite make eating or moving difficult. Ask for practical food and activity advice that fits your symptoms and any other illness.

5 Steroids and hormone replacement

Some immune effects need steroids or lasting hormone replacement. Follow the exact prescription and taper. Carry the relevant alert information, and do not stop or reduce these medicines yourself when anticancer treatment is held or completed.

Tests: what is required and what each result means

Safety tests, tumour tests and response scans answer different questions.

Before treatment

Full blood count, kidney and liver tests, glucose and thyroid tests; BRAF tumour testing and review of autoimmune conditions, transplantation, prior immune reactions and current medicines.

During treatment

Full blood count, kidney/liver tests, glucose and thyroid tests each treatment cycle. Cortisol and other tests are added when clinically indicated. Every visit also reviews bowel, breathing, skin, endocrine, neurological and other symptoms; a normal blood test does not exclude immune toxicity.

Scans and tumour markers

Ask the team to write the next assessment and who will arrange it. Bloods used to monitor kidneys, liver, hormones or muscle are not automatically measures of cancer response. There is no universal tumour-marker blood schedule for these skin-cancer regimens; a scan or extra blood test is requested for a specific clinical reason.

Getting the result

Keep a named contact for bookings and results. If an expected appointment or result is missing, call that service; do not take silence as confirmation that everything is complete.

What to expect between visits

You can feel well and still need the planned checks. New symptoms deserve attention even on a day without an appointment.

Your usual pattern

Record the start and change in symptoms, temperature if advised, bowel pattern, drinking, urine and the effect on normal tasks. A simple few-line record is enough. Use it to describe a change rather than trying to grade or diagnose a toxicity yourself.

Eating, drinking and activity

Report nausea, mouth discomfort, diarrhoea or weight loss before they make eating and drinking difficult. The team can adjust symptom support and arrange dietitian or rehabilitation advice.

Skin checks, sun protection and optional photographs

This complements clinical assessment and does not replace it.

Protect your skin

Protect exposed skin with clothing, a hat and shade. Use broad-spectrum sunscreen with sun protection factor (SPF) 30 or higher and reapply as directed. Avoid sunbeds. Cloudy weather does not remove ultraviolet exposure.

Know the pattern of your skin

Ask the team to show you how to check your skin and treated area. Use a mirror or a helper for areas you cannot see. Report a new lump, a changing lesion or a sore that does not heal.

Photographs are optional

A private, dated photograph can help describe a change if your team finds it useful. Keep the lighting and size reference consistent. Photographs support a discussion; they do not replace examination and need not be uploaded.

Wounds, nails and sensitive skin

Follow the wound and skin-care instructions for the treated area. Ask before adding a cream or remedy. Increasing redness, discharge, pain, bleeding or difficulty using the area needs assessment.

Pregnancy, fertility and protecting others

Discuss these precautions before treatment even if pregnancy is not currently planned.



Medicine-specific precautions

Planning ahead

Ask early about fertility preservation, sexual health and symptoms affecting intimacy. Tell the team about any chance of pregnancy, breastfeeding or a partner’s pregnancy so that the correct medicine-specific advice can be given. Keep the precautions and their end dates in the written plan.

Safe handling and supply

Do not share anticancer medicines. Keep oral medicines in their original labelled packaging, away from children and anyone who could take them accidentally. Return unused medicines to the supplying pharmacy and follow its handling advice. A treatment hold does not remove pregnancy precautions.

Questions worth asking

When are my next tests?

Full blood count, kidney/liver tests, glucose and thyroid tests each treatment cycle. Cortisol and other tests are added when clinically indicated. Every visit also reviews bowel, breathing, skin, endocrine, neurological and other symptoms; a normal blood test does not exclude immune toxicity. Your team adds the actual dates.

Can I use this without a smartphone?

Yes. The full handbook, three-page summary and wallet card are printable. The QR code is optional; keep actual contacts and written appointments on the paper copy.

> **Try explaining the plan back:** Can you explain your treatment setting, medicine schedule, next tests, the symptoms that need urgent advice and the number you would call?

Your diagnosis and treatment setting NCCP 00431a: advanced unresectable or metastatic melanoma in adults. This patient guide is the melanoma context, not the separate liver-cancer indication.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Combination induction

Active medicines: Nivolumab + Ipilimumab

Emetogenic risk: low

Medicine	Dose / route	When
Individual low-risk anti-sickness plan	Clinician/pharmacy completion if required	According to the local low-emetic-risk policy No routine immune-toxicity steroid or issued prescription is created by this draft.

Additional phase-specific support

- Combination induction is low emetic risk; nivolumab maintenance is minimal risk. Follow the individual/local anti-sickness plan. Routine premedication is not usually required; steroid immune-toxicity rescue is not routine prophylaxis.
- Discuss fertility and pregnancy precautions before starting. Immunotherapy may harm a pregnancy. Use the contraception plan and after-treatment duration specified for the medicines you receive; ask about breastfeeding restrictions. Tell the team promptly about possible pregnancy. Do not stop necessary hormone replacement or prescribed steroids without advice.
- Immunotherapy can cause inflammation in healthy organs during treatment or after it stops. Contact the oncology team promptly for new diarrhoea, blood in stools, cough or breathlessness, yellow skin or eyes, severe headache, vision changes, marked thirst, unusual weakness or a spreading rash. Tell any clinician that you are receiving or have received immunotherapy.
- This is not an issued prescription. The clinician and pharmacist must complete and authorise any supportive medicines.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Nivolumab alone

Active medicines: Nivolumab

Emetogenic risk: Minimal

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- Combination induction is low emetic risk; nivolumab maintenance is minimal risk. Follow the individual/local anti-sickness plan. Routine premedication is not usually required; steroid immune-toxicity rescue is not routine prophylaxis.
- Discuss fertility and pregnancy precautions before starting. Immunotherapy may harm a pregnancy. Use the contraception plan and after-treatment duration specified for the medicines you receive; ask about breastfeeding restrictions. Tell the team promptly about possible pregnancy. Do not stop necessary hormone replacement or prescribed steroids without advice.
- Immunotherapy can cause inflammation in healthy organs during treatment or after it stops. Contact the oncology team promptly for new diarrhoea, blood in stools, cough or breathlessness, yellow skin or eyes, severe headache, vision changes, marked thirst, unusual weakness or a spreading rash. Tell any clinician that you are receiving or have received immunotherapy.
- This is not an issued prescription. The clinician and pharmacist must complete and authorise any supportive medicines.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Hormones / head / eyes

Nivolumab / ipilimumab

New severe headache, visual change, unusual tiredness, dizziness, thirst or frequent urination may signal immune effects and needs prompt advice.

3 Heart / muscles

Nivolumab / ipilimumab

Chest pain, palpitations, marked muscle weakness or swallowing difficulty can be serious immune effects. Seek urgent assessment.

5 Kidneys / urine

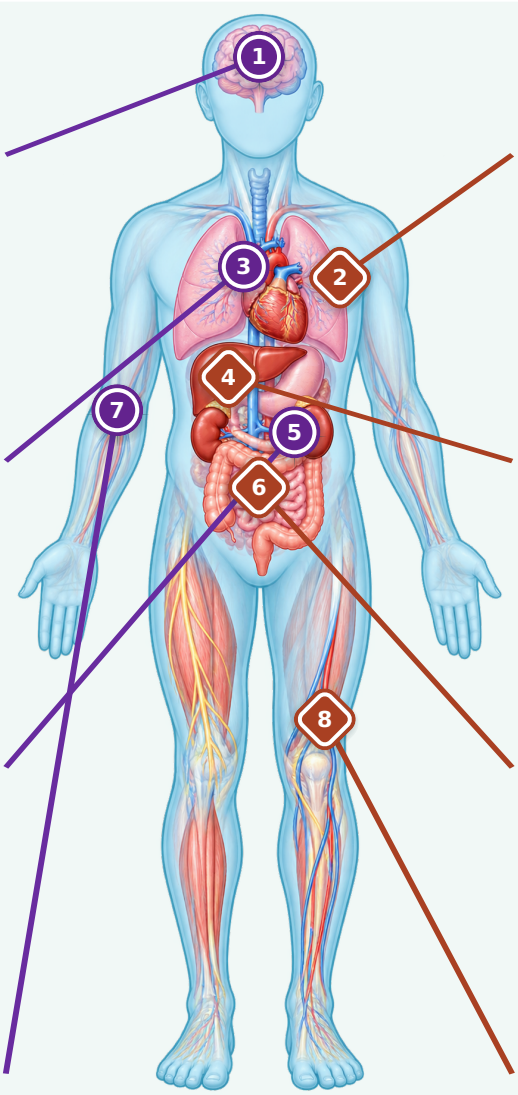
Nivolumab / ipilimumab

Kidney inflammation may be detected on blood tests. Report reduced urine, swelling, blood in urine or a marked change from normal.

7 Skin / mouth

Nivolumab / ipilimumab

Report rash or itching early. Blistering, peeling, mouth or genital sores, or rash with fever needs urgent assessment.



2 Lungs / breathing

Nivolumab / ipilimumab

New cough or breathlessness may reflect lung inflammation or another serious cause. Call promptly; severe breathing difficulty is an emergency.

4 Liver

Nivolumab / ipilimumab

Inflammation may first show in blood tests. Report yellow eyes or skin, dark urine, unusual itching or upper-abdominal pain.

6 Bowels / abdomen

Nivolumab / ipilimumab

Report increased stool frequency, diarrhoea, blood or mucus, or abdominal pain early. Severe pain, black stools or dehydration needs urgent help.

8 Nerves / strength

Nivolumab / ipilimumab

New tingling, weakness, unsteadiness, facial change or trouble swallowing needs prompt review. Sudden weakness or speech difficulty is an emergency.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Immunotherapy can cause inflammation in healthy organs during treatment or after it stops. Contact the oncology team promptly for new diarrhoea, blood in stools, cough or breathlessness, yellow skin or eyes, severe headache, vision changes, marked thirst, unusual weakness or a spreading rash. Tell any clinician that you are receiving or have received immunotherapy.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

SACTCHECK · CHAPTER 04

Sources and further information

Nivolumab and ipilimumab for advanced melanoma · National Cancer Control Programme (NCCP) 00431

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **NCCP 00431 v11 · pSee current source monitoring/supportive-care sections**
   https://healthservice.hse.ie/documents/6734/431_Nivolumab_1mg_Ipilimumab_3mg.pdf
2. **EMA: Opdivo medicine overview**
   https://www.ema.europa.eu/en/medicines/human/EPAR/opdivo
3. **EMA: Yervoy medicine overview**
   https://www.ema.europa.eu/en/medicines/human/EPAR/yervoy
4. **NCI: melanoma — patient information**
   https://www.cancer.gov/types/skin/patient/melanoma-treatment-pdq
5. **NCI: melanoma treatment — clinical evidence**
   https://www.cancer.gov/types/skin/hp/melanoma-treatment-pdq
6. **NCI: eating and nutrition during cancer treatment**
   https://www.cancer.gov/about-cancer/treatment/side-effects/nutrition
7. **NCI: palliative care during cancer treatment**
   https://www.cancer.gov/about-cancer/advanced-cancer/care-choices/palliative-care-fact-sheet
8. **NCI: skin cancer — patient information**
   https://www.cancer.gov/types/skin/patient/skin-treatment-pdq
9. **HSE SunSmart: protection from sun and sunbeds**
   https://www.hse.ie/conditions/cancer/protect-skin-sun-sunbeds/